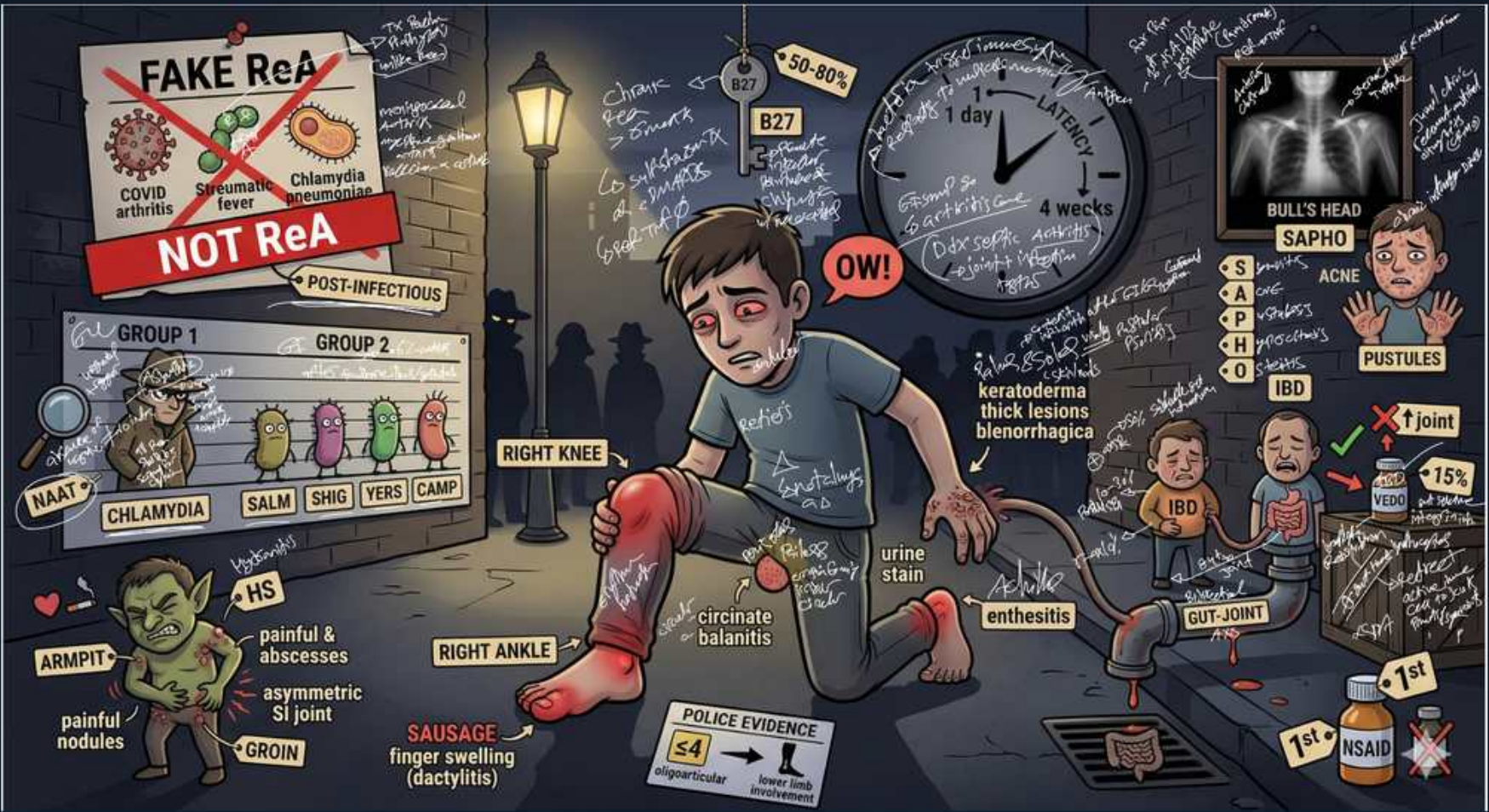


Spondyloarthritis — Reactive Arthritis, IBD-Associated Arthritis, SAPHO, Hidradenitis



1. FAKE ReA / NOT ReA

WHAT YOU SEE

Crossed-out poster FAKE ReA, NOT ReA with COVID, rheumatic fever, Chlamydia

WHAT IT ENCODES

True reactive arthritis follows a GU or GI infection at a DISTANT (sterile) site weeks later and is HLA-B27 associated. Distractors that are NOT classic ReA: acute rheumatic fever (post-Group A strep, has migratory arthritis + carditis + Jones criteria), post-streptococcal reactive arthritis, and viral/post-COVID arthritis. Also note Chlamydia CAN cause ReA when it triggers a distant sterile arthritis, but septic gonococcal arthritis (with organisms IN the joint) is a different entity.

BOARD TRAP

Migratory polyarthritis + carditis + recent strep pharyngitis is rheumatic fever (Jones criteria), NOT HLA-B27 reactive arthritis — picking ReA here is the trap.

2. Detective Group 1/2 bugs

WHAT YOU SEE

Lineup of bacteria: Chlamydia (GU) vs Salmonella, Shigella, Yersinia, Campylobacter (GI)

WHAT IT ENCODES

Two trigger groups: GENITOURINARY — Chlamydia trachomatis (most common GU cause; also implicated: Ureaplasma); and GASTROINTESTINAL — Salmonella, Shigella, Yersinia, Campylobacter (mnemonic 'Some Sneaky Yucky Critters'). The arthritis is STERILE/reactive — synovial fluid cultures are negative (cannot culture the organism from the joint), distinguishing it from septic and gonococcal arthritis. Treat the triggering Chlamydia infection (azithromycin/doxycycline) and the partner; GI ReA antibiotics do not change the arthritis course.

3. Goblin with nodules (HS)

WHAT YOU SEE

Goblin labeled HS with painful nodules/abscesses in ARMPIT and GROIN

WHAT IT ENCODES

Hidradenitis suppurativa: chronic recurrent painful inflammatory nodules, abscesses, and sinus tracts in apocrine-bearing intertriginous skin (axillae, groin, inframammary, perineum). It is part of the follicular occlusion tetrad and associates with spondyloarthritis, IBD (especially Crohn), obesity, and smoking. Treatment: topical/oral antibiotics, adalimumab (TNF inhibitor, FDA-approved), and surgery for sinus tracts.

4. Asymmetric SI joint

WHAT YOU SEE

Note: painful & abscesses, asymmetric SI joint

WHAT IT ENCODES

Spondyloarthritis-spectrum arthritis is typically ASYMMETRIC, OLIGOARTICULAR (≤ 4 joints), and LOWER-limb predominant, often with UNILATERAL/asymmetric sacroiliitis — contrasting with the symmetric bilateral sacroiliitis and symmetric small-joint disease of ankylosing spondylitis and RA respectively. Enthesitis (Achilles, plantar fascia) and dactylitis accompany it.

5. B27 + timeline 1 day–4 weeks

WHAT YOU SEE

Clock showing 1 day to 4 weeks, B27 tag

WHAT IT ENCODES

Reactive arthritis appears ~1–4 weeks (typically 2–4 weeks) AFTER the triggering GU/GI infection — by the time arthritis appears the infection may have resolved. HLA-B27 is positive in ~30–50% and predicts more severe, axial, chronic, and recurrent disease (and extra-articular features like uveitis). Most cases are self-limited (3–12 months); a minority become chronic.

6. Red knee

WHAT YOU SEE

Swollen RIGHT KNEE on the central figure

WHAT IT ENCODES

ReA causes asymmetric large-joint oligoarthritis of the LOWER limbs (knee, ankle most common). An acutely hot swollen knee mandates arthrocentesis to EXCLUDE septic and crystal arthritis — ReA synovial fluid is inflammatory (WBC ~2,000–50,000) but STERILE on culture and crystal-negative. First-line treatment is NSAIDs; intra-articular or short-course steroids and DMARDs (sulfasalazine) for refractory/chronic disease.

7. Sausage toe / dactylitis

WHAT YOU SEE

Swollen SAUSAGE finger/toe labeled dactylitis

WHAT IT ENCODES

Dactylitis ('sausage digit') — diffuse swelling of a whole toe/finger from synovitis plus tenosynovitis — is a shared SpA hallmark seen in reactive arthritis and PsA. Heel pain from Achilles enthesitis/plantar fasciitis is also classic for ReA.

8. Conjunctivitis / red eyes

WHAT YOU SEE

Red irritated eyes on the figure

WHAT IT ENCODES

Reiter triad of reactive arthritis: arthritis + conjunctivitis (or anterior uveitis) + urethritis — mnemonic 'can't see, can't pee, can't climb a tree.' Conjunctivitis is usually sterile and self-limited; recurrent ReA can cause acute anterior uveitis (painful red eye, photophobia) requiring ophthalmology referral and topical steroids. The full triad is present in only a minority — its absence does not exclude ReA.

9. Urine stain / urethritis

WHAT YOU SEE

URINE STAIN and circinate balanitis labels

WHAT IT ENCODES

Sterile urethritis (dysuria, discharge) can occur even with GI-triggered ReA (not only Chlamydia). Circinate balanitis — painless serpiginous/annular erosions on the glans penis — is a characteristic mucocutaneous lesion. Test for Chlamydia/gonorrhea (NAAT) when GU symptoms are present and treat the STI plus partner.

10. Keratoderma blennorrhagica

WHAT YOU SEE

Label keratoderma blennorrhagica, thick lesions

WHAT IT ENCODES

Keratoderma blennorrhagicum: hyperkeratotic, crusted, pustular plaques on the SOLES and palms — histologically and clinically INDISTINGUISHABLE from pustular psoriasis. Along with circinate balanitis and oral ulcers, it is a mucocutaneous manifestation of reactive arthritis.

BOARD TRAP

Pustular plaques on the soles in a young man weeks after dysentery/urethritis are keratoderma blennorrhagicum (ReA), not a primary fungal or pustular-psoriasis flare — the post-infectious context and arthritis are the discriminator.

11. Bull's head SAPHO X-ray

WHAT YOU SEE

Chest X-ray BULL'S HEAD labeled SAPHO with ACNE, PUSTULES

WHAT IT ENCODES

SAPHO syndrome = Synovitis, Acne, Pustulosis (palmoplantar pustulosis), Hyperostosis, Osteitis. The anterior chest wall sternocostoclavicular hyperostosis produces the classic 'bull's head' (steer/bull horn) appearance on bone scan/imaging. Related to chronic recurrent multifocal osteomyelitis (CRMO) in children. Treat with NSAIDs, bisphosphonates, and TNF inhibitors.

12. IBD gut-joint

WHAT YOU SEE

Figures labeled IBD, GUT-JOINT with up arrow joint, 15%

WHAT IT ENCODES

Enteropathic arthritis (~enteropathic SpA) complicates Crohn/UC. PERIPHERAL arthritis comes in two types: Type 1 (pauciarticular, ≤5 large joints, acute, PARALLELS bowel activity, self-limited) and Type 2 (polyarticular, small joints, runs INDEPENDENT of gut activity, chronic). AXIAL disease (sacroiliitis/spondylitis, HLA-B27 linked) runs COMPLETELY independent of bowel activity. ~10–20% of IBD patients have arthritis.

BOARD TRAP

Type 1 peripheral enteropathic arthritis improves when the bowel disease is controlled, but AXIAL (sacroiliitis/spondylitis) does NOT — assuming all IBD arthritis responds to gut-directed therapy is the trap.

13. NSAID caution

WHAT YOU SEE

Pill bottle 1st / NSAID with caution mark

WHAT IT ENCODES

NSAIDs are first-line symptomatic therapy for SpA arthritis, BUT in IBD they can trigger or worsen bowel flares and should be used cautiously/short-term. For active IBD with arthritis, TNF inhibitors (infliximab, adalimumab) treat BOTH gut and joints. Note IL-17 inhibitors (secukinumab) can WORSEN/precipitate IBD and are avoided in this population; sulfasalazine helps peripheral disease.

BOARD TRAP

Reaching for chronic NSAIDs in an IBD patient with arthritis can flare the colitis — when bowel disease is active, a TNF inhibitor is preferred over NSAIDs; IL-17 inhibitors are contraindicated because they worsen IBD.